

Relating to Clients: Interpersonal Attitudes

Though Vasudeva spoke not a word himself, the speaker fET him receiving his words into himself, quietly, openly, unhurriedly, missing nothing, not jumping ahead through impatience, attributing neither praise nor blame—just listening. Siddhartha fET what happiness can come from opening to such a listener, having one's own life, one's seeking, one's suffering enter this other's heart.

HERMANN HESSE

Our exploration of the Empowerment Therapy practitioner's mindset continues in this chapter. Here we turn our gaze outward to consider those attitudes that govern the way we engage with clients.

UNCONDITIONAL ACCEPTANCE

First and foremost, we accept our clients for who they are. If we don't, we cannot create and provide a therapeutic environment. Carl Rogers refers to this therapeutic condition as “unconditional positive regard.” I like to call it unconditional acceptance: understanding clients from their perspective without judgments or evaluations, positive or negative. This is to say that we neither approve nor disapprove of clients; we respect their views and value them for who they are, as they are. We remain calm and open to their experience, and we communicate this through empathic understanding. In a space of presence, empathy, and acceptance, clients are free to explore their inner life safely. They begin to see themselves more clearly, without the distortions stimulated by notions of unworthiness, e.g., “I am defective” or “I am not worthy of love.” They begin to face previously suppressed feelings and thoughts, thus gradually increasing self-acceptance. They begin to recognize that their worth as human beings is inherent, not contingent on performance.

Unconditional acceptance has an element of grace. We accept our clients' foibles even when they affect us. For example, if our clients lie to us, we accept them. If they come in late to sessions or cancel frequently, we accept them. If they get angry with us, we accept them. This does not mean that we don't address things that we consider important. It does mean that we do so without judgment and with warmth.

Unconditional acceptance is a tall order. There are times when we don't like what our clients say or do. When this happens, it's important to give it attention right away. We can engage in self-empathy to help us understand our unmet needs in the situation so that we can restore acceptance. For a client in an extramarital relationship, my internal monologue might sound like this:

Oh my goodness, I see a train coming down the tracks. I feel apprehension and concern mixed in with some judgments. What are my judgments? Let me let them all out. "What in the world are you doing?! Are you crazy?! Do you not give a hoot about your kids?" <sigh> I feel some release of tension.

What are my needs here? Oh, that's clear: everyone's well-being, but especially the children's. I think she's doing something very risky that's likely to lead to suffering not just for her but for everybody. I think it's important to express my concern.

But what is my need right now? Let me see. Contribution. I want to help protect the children. And what's my strategy? I'll talk to Viviana. I want to prevent suffering, or at least try.

At the next session, I would listen empathically to the client's reasons for engaging in such a relationship. Once we make empathic connection, I would say something like this:

THERAPIST: I have a concern. Do you mind if I express it?

CLIENT: No, go ahead.

THERAPIST: I think your relationship with Edgar could cost you your marriage, and if this happens, you are going to be in a very difficult situation and so are the kids. And it seems to me that you might not be aware of what might happen. How do you feel hearing this?

CLIENT: Oh, I know it's risky, but I'm being careful. I'm just tired of feeling numb. Edgar makes me feel alive!

The session continues like this. When the client has expressed all her reasons for engaging in this relationship, I might say:

THERAPIST: Thank you, I have more clarity about the needs that this relationship meets and how fulfilling it is to you. And still, I'm concerned. Would you be willing to talk about potential risks?

The spirit out of which we speak is concern, not preaching or moralizing. That said, it's important that clients' aims be ethical if we are to extend unconditional acceptance. It's not possible to embody this therapeutic attitude when clients report meeting their needs at the expense of others without regret. Regarding the boundaries of unconditional acceptance, Barbara Temaner Brodley says:

The therapist should not take on a client whose aims are immoral unless [the therapist] clarifies [their] willingness to work with the client in the hope that he will change his aims. [...]

The factor of *unconditional positive regard* can only be met for clients whose values are humanist and democratic and truth-seeking. (*Practicing Client-Centered Therapy*, p. 43.)

If a client shows no regret about battering his wife, for instance, I can still work with him, but I will let him know that I'm going to attempt to change this value. If he doesn't want this, I will refer him to another therapist.

EMPATHY

Empathy is the capacity to understand another from their own frame of reference. When we hear someone empathically, we attempt to perceive their intended meaning by focusing on their feelings, needs, and wishes, as well as the significance of the issue at hand and their relation to it. The benefits of empathy in therapy are well known. When we listen to clients without judgment and endeavor to understand with our minds and hearts, we support them in shifting from reporting problems to processing experiences. And, of course, it's in live processing that healing occurs.

Identifying Feelings and Needs When listening empathically, NVC invites us to focus not just on feelings but also on needs. This focus helps us understand multiple layers of meaning in what clients say about their challenges. When clients talk about internal conflicts, which is most of the time, we capture the needs on each side of their dilemmas. For instance, a client reports that he is unhappy at his job because the demands are excessive and he is very stressed out and anxious, but it pays well so he doesn't want to leave. We acknowledge the client's needs for emotional well-being and peace of mind on one hand, and the need for security on the other. I have found needs-based empathy to be NVC's most powerful contribution to my practice. When I began to reflect needs to my clients, they gained deeper self-understanding, clarity, and confidence.

Guessing Wishes In addition to feelings and needs, sometimes we try to understand clients' implicit wishes. In all cases, we check our understanding by asking clients if what we understand is what they mean. In the following example, put yourself in the place of the client. Notice if you experience any difference between the empathic reflection on the left, which focuses only on feelings, and the one on the right, which goes further to center needs and guess the client's wishes.

CLIENT: Matt is always working. Even when he comes home at night, he has dinner, goes to his office, and continues to work. We hardly talk.

THERAPIST:

Empathic Guess without
Needs and Wishes

I'm hearing that you are
feeling lonely, is that it?

Empathic Guess with
Needs and Wishes

I'm hearing that you're
lonely, needing
connection and
closeness. You wish he
would spend more time
with you, is that it?

Understanding Meaning We gain empathic understanding by focusing on clients' meaning, in addition to the feelings, needs, and wishes they express. We look for the significance of every sentence or cluster of sentences. What are they trying to convey? What are they implying? A useful question to ask ourselves is, "Why is this important to him or her?"

CLIENT: There will be more layoffs. The writing is on the wall.

THERAPIST: Are you afraid you'll be next?

CLIENT: My son has a girlfriend now. He's totally focused on her.

THERAPIST: Are you sad that you don't spend as much time together as you used to?

CLIENT: Fall is here. You know what that means.

THERAPIST: Are you dreading winter?

CLIENT: She was a good therapist, but she thought I should divorce. I don't believe in divorce.

THERAPIST: You want to work with a therapist that understands and respects your values. Is that it?

Elements of Empathy

In graduate school, we learn to listen to clients and capture their feelings and the meaning of their words, but as we know, empathy is more than cognitive understanding. ET offers a concrete approach to help us embody empathic presence. These are the elements.

Preparation We prepare for empathic listening by engaging in a practice that promotes self-connection. For me, it is meditation.

Presence and Focus When the client talks, we listen calmly, with a silent mind and an open heart. We set aside our own concerns temporarily. When we are fully present with a client during a session, only he or she exists. We pay attention to the client's feelings, needs, meaning, and their implicit, unspoken wishes.

Present Moment We try to connect with clients' feelings and needs *in the present moment*, not how they must have felt when the situation occurred last week or when they were children. We flow with clients. They may have been angry when they began talking about a problem, but 20 minutes later, they are afraid. We don't go back to anger; that feeling is gone. By the same token, if we sense that clients are afraid but they are just expressing anger, we stay with anger. We don't ask if they are afraid. Clients lead and we walk next to them—not ahead or behind.

Empathic Guesses We check our understanding frequently by telling clients what we understand, and we invite the client's correction of our understanding. We might preface a guess with "This is what I'm getting. . . ." or "If I'm with you. . . ." Or we might finish with "Am I with you on this?" or simply "Please refine my understanding."

It's critical that we understand our clients accurately and recognize them as the authority on their own subjective experience. This is to say that the client, and only the client, knows what is true for him or her. Consequently, our empathic responses are *always tentative*. I can't tell you how often I think I understand "exactly" what the client is saying, only to discover that I don't. Similarly, when I express myself freely with fellow therapists and then ask for an empathic reflection, most of the time, I need to correct or refine their understanding of my meaning.

Dream Metaphors When using metaphors in empathic reflection, I propose that we try to capture the client's dream, as opposed to their nightmare. The dream connects with grief, whereas the nightmare, even if accurate, may reinforce fear, dejection, and hopelessness. To be clear, we're not trying to put a positive spin on the client's situations. We are trying to capture the longing of their heart—the need—in the form of a metaphor. Pretend that you are the client, and notice if you feel a difference between these two empathic guesses, the nightmare metaphor on the left and the dream on the right.

CLIENT: All my friends are married. They have a partner to live life with. And they all have babies. And here I am, alone, just watching.

THERAPIST:

Nightmare Metaphor

I'm hearing that you're sad and lonely because you don't have a life companion. And you long to have a family. *It's as if you're outside the house, looking in.* Is that how you feel?

Dream Metaphor

I'm hearing that you're sad and lonely because you don't have a life companion. *And you long to have your own nest and fledglings.* Is that how you feel?

Here is an example from my own life:

I was talking to a therapist friend of mine about a problem in a relationship. I said that I had asked certain things over and over, to no avail. He said, "I'm guessing that you feel like a woodpecker beating your head against a tree." He was right! That was exactly how I fET. At that moment, my need for empathic understanding was well met. However, the image stayed with me for a long time, and every time I thought of it, it evoked discouragement and a sense of being stuck. Noticing this, I did an experiment: I brought to mind the problematic situation and noticed my emotions. I fET frustrated and irritated. Then, I brought to mind the woodpecker image and I fET discouraged and stuck, again.

Offering dreadful images is particularly risky when we are working with clients who have suffered trauma. I propose that we avoid nightmare metaphors so we don't compound their anxiety-provoking imagery.

Attunement This refers to our awareness of and responsiveness to clients' feelings, needs, and wishes, whether expressed directly or indirectly, verbally or nonverbally (through tone of voice, volume, shallow breath, a facial twitch, a sigh, looking down, and the like). Attunement stems from and goes beyond empathy, as we sense clients' experience unfolding and changing moment by moment and respond accordingly.

During a session, a client asks for the therapist's input about a new relationship. They had talked about this before, and she suggests that he trust his intuition about

this person. He says OK, and his eyes glaze over. She asks, “How does this sound?” He replies that this is hard to do because he doesn’t trust his intuition. The therapist learned something new about the client because she was attuned to his body language.

Tracking Sometimes clients go down rabbit holes, and their original point is easy to lose. In order to stay connected, it’s important to keep track of where they are headed and what they mean to communicate. This requires mindfulness.

CLIENT: It’s hard to be a father. I mean, I want to support my kids’ choices, but sometimes it’s hard. Alexandra came home from math tutoring the other day—she does the tutoring—and said, “Dad, I don’t want to go to college. I can’t stand four more years of dog obedience training.” She’s a bit of a rebel, you know. I think her friend Genna is a bad influence. When she started hanging out with Genna, we found pot in her nightstand drawer.

THERAPIST: Mm. It’s important to you to support your children in finding their own paths, but you want to protect them from choices that might not serve them well. How to strike a balance. And, that’s the dilemma you’re facing with Alexandra right now. Am I with you?

The client expressed a conflict and then took a couple of detours. It would have been easy to lose the main point in this long exchange because he brought up other issues that concerned him—his daughter’s friend and her use of pot. Keeping the beginning of this exchange in mind, the therapist captured both sides of the conflict in the universal form first—a value—then in the personal form—the practice.

Duration As you know, we remain empathically present and refrain from interventions for as long as the client’s experience is unfolding. As we do this, clients access greater and greater depth. They may discover that the crux of their problem is not what they thought it was originally. For example, a client may talk about getting drunk on weekends. They may see the problem as their lack of self-control. If we remain empathically present, they may uncover and connect with loneliness and emptiness in their lives. Had we rushed to discuss ways to enhance self-control, the client wouldn’t have realized something very important.

This dynamic is particularly risky with clients who suffer from addictions. We may focus on the addiction without making space for the underlying pain to emerge. I propose that we give clients ample space, intervening only when they are done expressing distress and releasing pain, and have come to an organic halt. To reiterate, we offer clients our input only after we have listened empathically for as long as it takes them to empty their mind and heart about an issue, and they come to a natural halt. This can take two thirds of the session or longer. You can review an illustration of this concept on Part Three, Session 1.

Therapist Input When making an empathic guess, we make sure not to add or mix in our own input. Such a mixture is likely to create confusion for clients about their own experience. On rare occasions, when something in the client’s narrative might be causing them harm, we may think it’s important to express something from our own perspective *during empathic listening*. In such situations, asking the client’s permission to offer input

makes it clear that it is coming from us, rather than being an empathic reflection. After making our point, we return to empathic presence.

THERAPIST: I want to say something about that. Do you mind?

CLIENT: No, go ahead.

THERAPIST: When you say that you deserve punishment, I feel concerned. I get that you feel regretful about leaving the kids alone and that you want to learn from this experience. But I think that thinking you deserve to be punished is debilitating and contributes to depression. How do you feel hearing this?

CLIENT: I see your point. I learned my lesson. Oh my goodness, I don't need to be punished. I have enough pain.

THERAPIST: Yes. That's all I wanted to say.

CLIENT: On top of it all . . .

Universal vs. Particular If clients talk about a situation in a universal way, we reflect our understanding that same way. And if they talk about a universal situation as if it pertained to them only, we check our understanding in the same way; we don't normalize it *at that moment*.

CLIENT: So many women go through sexual abuse. It's unbelievable!

THERAPIST: I'm hearing that it's shocking to you to know how prevalent sexual abuse is among women. How much you want women to be safe in the world and to be treated with respect and care. Is that it?

Notice that although the client might have suffered sexual trauma, the therapist didn't redirect the focus to the client's own experience. We also follow the lead of our clients when they focus on the particular.

CLIENT: It really bothers me when my sister calls me names. I don't know why.

THERAPIST: It affects you very much when she calls you names. I'm guessing you want to be treated with respect, is that it?

Notice that the therapist didn't immediately generalize by saying "Everybody needs respect." This response would likely take the client from experiencing into thinking and could thus limit the depth of their exploration. Voicing needs, in and of itself, has a humanizing, normalizing effect. If clients need further help realizing that their reactions are common and prevalent, there is time enough to offer such a perspective after they are done exploring a particular theme.

Self-Monitoring When we are in empathy, we talk less than we normally do, our voice is softer, and our pitch is lower. These are some things I observe that let me know whether or not I am in the field of empathy.

Anchoring Experience When listening to clients, I pay attention to the situation they are talking about and then briefly bring to mind a similar situation in my own life. I find this practice very useful because it helps me understand clients from within. Most of the time, I

can relate to their distress firsthand. That said, I keep in mind that this is *my* experience, and I remain attentive to theirs.

Guilt, Shame, Regret, and Remorse When clients express guilt, shame, regret or remorse regarding an action of theirs, it's important to recognize and acknowledge the needs that they did not meet *first*, and then the needs that they were trying to meet. For instance, "I get that you regret getting drunk last night because you value moderation and self-care. And I imagine that you needed relief and relaxation. Is that so?" In an effort to help alleviate guilt and shame, we might be quick to help clients recognize the needs that they were trying to meet by a given action. However, if we fail to acknowledge the value and need that give rise to their guilt and shame, clients might think that we don't understand their values, or that our values differ from theirs to the point that we're not a good fit as a therapist.

Roadblocks to Empathy

Clinical Thoughts and Interventions Many thoughts and actions that are inherent to therapy can move us out of empathy when listening to clients.

- ◇ Assessing their mental status
- ◇ Diagnosing them
- ◇ Thinking of what the intervention should be
- ◇ Asking questions beyond those that are clarifying
- ◇ Offering insights, education, or perspective
- ◇ Challenging cognitive distortions

It's a matter of timing. Are we listening empathically or doing something else internally? (Multitasking?) I propose that we pay attention to this issue because it matters.

Periodically, I ask clients if therapy is helping them, and if so, what is most helpful. Over 30 years of practice, these are the answers I have heard most frequently: "to be heard," "your empathy," "that you understand." No one has ever said, that I can remember: "your knowledge," "the things I've learned," "your insight," or "your advice." The thoughts and actions above are important, but not *the* most important thing to my clients, per report. If empathy is so beneficial, I want to be clear when I am and am not offering it.

Personality Disorders Diagnosing clients is already on the list of empathy-blocking thoughts, but personality disorders merit special mention. For instance, rather than receiving and trying to connect with a client who is in pain, I may think of yet another of her borderline crises. Instead of witnessing a client's need for recognition and to be seen, I may think of his words as typical of the self-aggrandizing narcissist. I fear that clients won't make much progress in therapy if we view them in this light. This approach gives the following idiomatic expression a new meaning: "What you see is what you get." If we see a

client as a narcissist, our interactions with him or her will likely reflect our view and we will reinforce those traits unwittingly.

Burnout Fatigue and exhaustion impact our ability to be and remain empathically present to our clients. This is another important reason to practice self-care.

Analysis Analyzing clients in our minds, drawing connections to the relationship with their parents and other past experiences, and attributing our own theoretical meaning to their actions as they speak blocks empathy.

Hearing Criticism We remain empathically present even when we are the target of criticism, blame, and other such forms of communication. We establish empathic connection before we respond or intervene.

CLIENT: You're trying to hurt me, like everyone else! No, I'm not going to take the stupid medicine. You're trying to poison me!

THERAPIST: I'm hearing that you're angry because you can't trust me or anybody. You want to be safe, and you will not take the medicine. Right?

CLIENT: That's right. You people are mean. You sent the police to my place the other day. You're trying to kill me!

THERAPIST: You can't trust anyone at this center. Are you angry because you don't feel you're safe here?

CLIENT: I'm *not* safe here. You sent the police.

THERAPIST: I'm guessing seeing the police at your place was scary. Was it?

CLIENT: Damn right, it was scary!

THERAPIST: Very scary not to feel safe in your own house.

CLIENT: I'm going to leave now.

THERAPIST: May I say something before you leave?

CLIENT: What?

THERAPIST: I understand this is hard to believe right now, but please know that I care about you. I asked the police to check on you because I was worried about your well-being. You didn't answer the phone for three days, and I didn't know if you were OK. How do you feel hearing this?

CLIENT: I don't believe you.

THERAPIST: Is it OK if we talk tomorrow for a half hour to touch base?

CLIENT: I don't know.

THERAPIST: Why don't we meet tomorrow at 2:30, and we can talk some more. I want to hear what you have to say. Is that OK with you?

CLIENT: OK.

It's comparatively easy to remain empathically present in the face of criticism and blame when a client is undergoing a psychotic episode. It's more difficult when clients are perfectly stable and they are critical of us. But the task remains the same: empathic connection before intervention or explanation. This applies to clients' relatives, as well. When we lose a client's relative, we are likely to lose the client.

Distractions When we are unable to remain present, we give attention to our thoughts and needs. This is an internal process.

I can't stand hearing that her little girl cries so hard every morning when her dad drops her off at the daycare center! . . . What is it? . . . Oh, yes, it's my love of children. I don't want them to suffer. I want them to be happy. <sigh> I can listen now.

Crisis Intervention

Empathy can be very helpful in a crisis or when there is imminent violence that we cannot prevent through the protective use of force. If a client is angry and yelling, empathy can help prevent violence. The following is an example of a situation in which empathy prevented what would have been a distressing and probably traumatic outcome for a client.

I was in my office at the community mental health center where I worked. Suddenly, I heard a man scream "Leave me alone!" He swore and cursed at the top of his lungs. I waited for a couple of minutes thinking that this would pass, but it continued. I rushed to the office where the screaming was coming from. There was a crisis intervention worker watching the door. "May I go in?" I asked. "It's OK," she replied. "There's a crisis worker in there already." I waited for a few minutes, but the screaming went on. I asked again if she would let me in as I knew a form of communication that could help. She allowed me to go in, finally.

In the room, there was a young man, about 17 years old, along with a psychiatrist, the young man's father, and a crisis worker. I sat on the floor and said, "I'm guessing you're angry because you want to be treated with respect, is that it?" The young man was suddenly quiet. "You desperately want to be heard, yes?" Silence.

I asked everyone if they would leave so that I could connect with the client. The crisis worker expressed concern for my safety, but everyone complied with my request. After continued attempts to connect and ample silence, the young man responded with monosyllables a couple of times. I said I guessed that he wanted to make his own decisions, a colloquial way of acknowledging his need for autonomy. After a while, I said I was worried that if he continued to yell he would be taken to the hospital forcibly. (I learned later that this was precisely what the psychiatrist was going to do.) I said: "I'm afraid. I don't want you to go through something like that. I care about you." He looked at me, and his eyes told me that we had connected.

I asked the doctor and father to come back in the room, and the visit resumed without further screaming. The client's father called the mother and asked her to come over to the center. I saw them walk down the hallway and out the door, one parent on either side of the young man. I was tearful and at peace.

Notice that in this situation I

- ◇ entered the scene with an open heart and without fear.
- ◇ intended not to de-escalate the client but to understand him.
- ◇ reduced the adult/adolescent, therapist/client power differential by sitting on the floor while the client was sitting on a chair.
- ◇ trusted the client and asked to be in the room with him by myself (I realized that I was taking a risk but chose to trust my intuition).
- ◇ made empathic guesses.
- ◇ allowed ample silence.
- ◇ intervened only when we had connected *and not before*. I raised the client's awareness of the risk of forced hospitalization if he continued to scream and curse.
- ◇ expressed my concern as one human being to another. I made myself vulnerable—I expressed care and fear about his well-being.
- ◇ trusted the power of empathy and vulnerability in a crisis.

A Peril of Empathy

Sometimes people interpret empathic understanding as agreement, which, of course, it is not. A client may feel emboldened to continue a harmful pattern because the therapist understands her or his actions. For example, once I agreed to meet one time with a man in my hometown who was having a conflict with his wife. I listened empathically and gave him a referral. Later on, his wife said to me that he referenced our meeting when trying to get his way. "Myra agrees with me. You should listen to me!"

Drawing from that experience, I now advocate for the other person when I see risk. So, if I were to meet with this man again, after connecting empathically, I would say, "I'd like us to take a look at Ernestina's needs now. I think that when you raise your voice in an effort to be heard, she may be scared, needing safety and respect. I propose we explore strategies in which you can be heard and Ernestina's need for respect can be met."

Practice

In response to the expressions below, I invite you to guess a feeling and two universal human needs, choosing from the following list: peace, trust, support, ease, beauty, mourning, collaboration, help, meaning, integrity, consideration, honesty, empathy, cooperation, touch, affection, harmony, order.

1. I come home from work. I cook, clean, do homework with the kids . . . Aaaaah!
2. Today we were told that there will be layoffs next month.
3. I think my girlfriend is cheating on me.

Take It to Your Practice

During a therapy session, notice if you are thinking while your client is speaking. If so, notice your thoughts and let them go.

If you are not doing it already, identify and express what you think your clients' needs are when you listen to them. Do so in the form of a question, as opposed to a declarative statement, to give the client a chance to correct or refine your understanding.

Invitation to Reflection

How do you define empathy?

In your view, what are some necessary elements of empathy? For instance, nonjudgment.

How do you think we can deepen our empathic presence?

WITNESSING

Often, our clients go through situations that lack any semblance of care or fairness. It's important to be a caring observer of what they go or have gone through and acknowledge it. I call this practice "witnessing." It lets clients know that they are not alone in their perception, that we not only understand their experience but *see it* for what it is. And we care. The challenge is to witness clients' experience without fueling anger, indignation, resentment, or outrage. If we are angry about what we are hearing, our choice of words, tone of voice, or body language will betray it. So that's not the time to witness. First, we work through our anger, internally. Once we restore calm and equanimity, we may say something like "I see that the working conditions at your company are not humane." We avoid calling the conditions "inhumane." The words inhumane and not humane sound similar but they evoke a different response in the client. To experience the difference, you may want to say each statement below out loud, listening as if you were the client, and notice if you have a different emotional response:

I see that the working conditions at your company are not humane.

I see that the working conditions at your company are inhumane.

The first statement likely connects the client with grief, whereas the second one can stimulate anger. Thus, we express the dream, albeit in its absent state, and not the nightmare.

The timing of witnessing is important. As always, we want to establish empathic connection first. We wait until the clients are done expressing themselves about the situation at hand; we watch for an organic shift, a noticeable easing of tension manifest in tone of voice and body language.

Witnessing also entails remaining present to clients as they recount their story over and over again. If we are fully present and if we are listening empathically, it is likely that each time they talk about their pain, healing is taking place. This process is a gradual one that can last a long time depending on the severity of the trauma, in addition to other factors such as temperament, internal resources, and support systems.

GENUINENESS

Genuineness is the term I use to refer to Rogers' attitude of congruence. I'm defining genuineness as speaking and acting in ways that are in harmony with our thoughts and emotions. When we are genuine in the therapeutic relationship, we are truly and deeply ourselves. I do not mean that in the name of genuineness we reveal our thoughts and reactions to our clients freely. No. Our words require discernment. We speak carefully, informed by therapeutic and ethical values, and always in the service of the client, i.e., not for the sake of genuine self-expression.

Similarly, genuineness requires us to be brutally honest with ourselves. If we dislike a client or experience anger, fear, revulsion, and the like, it's important that we admit this to ourselves. If we suppress disturbing emotions, we could act on them unconsciously and do harm. When this happens, I propose that we engage in self-empathy, uncover our unmet needs, and seek consultation. A word of caution: when I was working at a community mental health center, I sought consultation regarding a client about whom I had a judgment. My supervisor at the time joined in judging my client. This was not helpful. So, I propose we seek consultation from colleagues who will help us resolve nontherapeutic thoughts and emotions and restore unconditional acceptance.

WARMTH

Warmth refers to our unspoken affection for clients, conveyed by our eyes, smile, and tone of voice. Warmth is a natural expression of positive regard, and positive regard—unconditional acceptance—is the foundation of safety. Clients require sufficient safety to move beyond their well-worn, emotional survival strategies and consider new possibilities. Warmth conveys to clients "You are safe with me." Clients are attuned to our emotional tone, and they perceive our warmth or lack thereof. Warmth can augment clients' satisfaction, and strengthen the therapeutic alliance.

One way to cultivate warmth is by treating ourselves affectionately. I discovered this when I was having conflict with a supervisor at a community mental health center where I worked. Every morning driving to work, I was in pain. To increase my fortitude, I would give myself a good dose of affection on the road. I touched my cheeks tenderly with the palm of my hand, and comforted myself with soothing, loving words. To my surprise, every time I did this, I treated my coworkers with increased warmth and affection.

CARE

Matteing Care is our interest in, and genuine concern about clients. Care is therapeutic because it lets clients know that they and their needs matter to us. Much of clients' suffering is not because their needs are not met, but because their needs are not treated by themselves and others as if they mattered. Conversely, treating needs as if they matter is nourishing in and of itself, whether or not those needs are actually met. When therapists show care about clients and their needs, clients internalize that they matter and that their needs matter. That matteing is a prerequisite for caring for themselves and endeavoring to build fulfilling lives.

Trust Care also fosters trust in the therapist. When clients perceive that we care about them, they can trust that what we do is with their well-being in mind.

Engagement When clients see that we care for them, they will know that what they do or don't do matters to us. Thus, they are more likely to engage in the therapeutic process and goals, and this leads to better outcomes.

Expression We don't express care so that clients know they matter to us. Instead, care comes through in our interactions naturally. We express care during a session with spontaneous, genuine exclamations, like "oh" when clients disclose something painful to them; when we return phone calls the same day or the next day; or when we call clients to hear how they are recovering from surgery.

Compassion I make a distinction between care and compassion. I understand compassion as experiencing pain when hearing about the suffering of others and wanting to do something that could alleviate their suffering. Naturally, if we are listening to our clients with an open heart, their pain will affect us. But it's not helpful to give free rein to compassion *during* sessions. Our pain can scare clients and limit their freedom of expression. A client of mine said to me, "When I told [a previous therapist] my story, her eyes watered. I told myself, 'Boy, I must be really screwed up!' I fET like saying, 'Are you OK?'" When we experience pain regarding clients' suffering during a session, we notice it, let it go, and return to empathy. We take time to connect with our own sadness *after* the session. That said, we cultivate compassion as an important quality of being a therapist and as part of our professional development.

RESPECT

Respect, as I define it, is treating others with courtesy, consideration, and thoughtfulness.

Punctuality Respect includes being on time for sessions, and if we are late on rare occasions, apologizing and explaining the reasons for the delay.

Confidentiality Clearly, we are careful about protecting clients' private information based on ethical principles and a legal dictum. This is fundamental. However, I have known of

therapists who talk with their partners or friends about their clients' situations, albeit anonymously. I consider this lack of respect for clients' privacy and a breach of trust. I realize that sometimes we need to vent and receive support, but I propose that we seek colleagues for this purpose, people who are also trained on the importance of confidentiality and whom we trust are not going to judge our clients or joke about their situations.

Apparel Regardless of how informally our clients dress, we wear work clothes. Working with marginalized, mentally ill clients, I want to dress the way I would if I were working with highly educated, affluent clients in positions of power. Hopefully, I would not overdress for this group, conversely.

Low Income Clients Throughout my career, I worked at centers serving people who had limited financial resources. The services were subsidized by grants, and clients received therapy at very low or no cost. Sometimes, clients were not treated with respect. As you can imagine, their options for finding services elsewhere were limited, leaving them not just impacted by such treatment but also with little recourse in finding effective support.

Mandated Clients Mandated clients are those referred by governmental agencies for driving under the influence of a substance, beating a partner, or abusing children. I've seen therapists turn into law enforcers with these clients, taking on a punitive approach! Respect can go down several notches with mandated clients.

Conditions and Other Matters When talking about clients' conditions, we want to protect their dignity by not conflating their identity and diagnosis. So, instead of saying "Wes is ADHD," we would say "Wes has ADHD." The same is true when we are discussing their insurance. Rather than saying "Patricia is Public Aid," we would say "Patricia's insurance is Public Aid" or she "has Public Aid." This way of speaking helps us remember that first and foremost, the client is a human being, independent of other circumstances.

VULNERABLE HONESTY

In Nonviolent Communication, we define honesty as offering a noninterpretive observation, revealing our feelings and needs vulnerably, without criticism or blame, and asking for what we want without making a demand (on demands and requests, see Chapter 5). It's important to express ourselves honestly when we have a concern regarding clients to be able to remain open to them, especially if they do something that we don't appreciate. If we don't address the issue, we run the risk of building resentment, impeding unconditional acceptance, and damaging the therapeutic relationship.

Addressing Sensitive Situations NVC's format for honest expression helps us address delicate situations with clients without a trace of criticism, thus protecting their emotional safety.

At an agency where I used to work, a therapist expressed frustration that at the end of a session, a client of hers continued to talk for 10 minutes or longer, regularly. She

said that she had expressed her desire to end on time, but the client didn't listen. I knew this therapist well and appreciated her sensitivity and care. I understood her dilemma, not wanting to force the client to leave.

In situations like this, the NVC model of pulling together the observation, feeling, need and request can help us to talk about sensitive topics without judgment, coercion, or power-over interactions.

Maureen, I'd like to talk about the end of sessions. The past two sessions you continued to talk for about 10 minutes after 7:00. I'm feeling concerned because I need space and rest between sessions. I like taking a break and doing some things before the next session. How would it be if I let you know when we have 10 minutes left and you start wrapping up so we end at 7:00? Would this help, you think?

Notice that the therapist made herself vulnerable by revealing her feelings and needs. Rather than "setting boundaries," she appealed to her client's care from one fellow human being to another.

Reducing the Power Differential By expressing our feelings and needs, we reduce the power differential. It's no longer the client only who is vulnerable; we are too.

Clarifying Our Purpose By ending with a request, we make it clear that our purpose in revealing our feelings and needs, disclosing how the situation impacts us, is to seek a solution, not to stimulate guilt and shame.

Therapists' Humanity In some approaches to therapy, there is a barrier between therapists and clients that prevents them from experiencing the therapist's humanity. Vulnerable honesty offers a model in which the therapist can be both human and vulnerable and still care for the separation of roles.

With vulnerable honesty, we can also share things about ourselves judiciously that help clients feel that they are not alone in their struggles, that we have limitations just like they do, or that we are coping with a challenge in a way that is inspiring to them. A more human-to-human connection that includes vulnerable honesty can contribute to healing in ways that are more difficult in its absence.

Practice

Assume that the following situations apply to you. How would you approach your client making yourself vulnerable?

- ◇ Your client doesn't pay for the service at the end of the session, even though you have requested this twice.
- ◇ Your client has cancelled sessions the same day three times in a row.

Take It to Your Practice

When you make a request of a client, preface it with your needs.

In couples and family therapy we can share with clients a way to express themselves honestly without criticism or blame, thus increasing safety and respect in primary relationships.

Invitation to Reflection

Do you think that the NVC honest expression format can benefit your therapy practice? If so, how?

PARTNERSHIP PARADIGM

The partnership paradigm invites us to engage with clients collaboratively. Therapist and client become a team of two. Nondirectivity is at its heart. I am grateful to Carl Rogers for introducing the nondirective attitude to psychotherapy practice. I have benefited from it greatly because my therapist adheres to it strictly. Years ago, when our work began, I suffered from self-doubt, a legacy of trauma. Marge's utmost respect for my autonomy was greatly empowering and, over time, it enabled me to take the reins of my life with determination and resolve. I learned to listen to my intuition and trust myself. And when I make mistakes, I trust that I will be able to correct them. Marge calls this "the paradox of empowerment through nondirectivity." Barbara Temaner Brodley describes the nondirective attitude as

... an attitude in which one is not intending to manage the affairs of others, not intending to give guidance, nor intending to conduct, regulate, command or control others. Clearly, the client-centered nondirective attitude is not a paternalistic attitude and it is an anti-authoritarian attitude [. . .]. It asserts the values of respect towards and trust in persons, egalitarian and democratic values, and the value of freedom understood partly in terms of John Stuart Mill (1859) ' . . . that of pursuing our own good in our own way, so long as we do not attempt to deprive others of theirs, or impede their efforts to obtain it.' (*Practicing Client-Centered Therapy*, p. 48.)

Respecting Clients' Meaning Practicing nondirectivity, we try to understand clients from their own perspective without offering additional meanings for their experience. We defer to the client as the ultimate authority of their inner experience.

Asking Permission At times we may offer an insight or psychoeducation. When we do, we ask the client's permission to offer our input, and do so in the spirit of collaboration and not as the therapist imparting knowledge and wisdom to the client from the top down. We might say, for example, "I have some thoughts about this. Do you want to hear them?" When we don't feel genuinely inclined to ask permission in some way, we offer our input as tentative.

Answering Questions Nondirectivity also means answering clients' questions and not turning the questions back on them, as in the classic "What do *you* think you should do?" When we answer a question, we do so carefully by saying "What I might do if I were in that situation is. . . ." When a situation is delicate—e.g., "Do you think I should marry him?" or "Do you think I should take the job and relocate"?—we might add "This is what I might do, but the situation is delicate so I don't want to influence your final decision. I suggest you talk to other people you trust, and I can help you connect with your intuition." Nondirectivity makes clients the ultimate authority of their own lives. As a result, they become stronger, more confident, and they take the reins of their lives.

In the following table, I contrast an authority paradigm with ET's partnership paradigm. Although presented separately for clarity, I see the attitudes as a continuum along which most of us move back and forth. This understanding allows me to look at myself with honesty and compassion. My goal is to embody the attitudes of the partnership paradigm more and more, while embracing my humanness when I don't.

AUTHORITY PARADIGM

Therapist as the Authority

Therapists regard themselves as the authority in the relationship. Symbols that bespeak this difference in status include asking clients to address them by their title, using a chair larger than that of the client, or meeting with clients from behind a desk. They share their knowledge as the experts in the relationship and in the spirit of education. When clients have not scheduled appointments for a while, therapists call them to schedule a session, expressing that they deem this to be good for the client. Therapists' ultimate goal is to help clients thrive.

PARTNERSHIP PARADIGM

Therapist as a Team Member

Therapists see themselves as partners in a team of two. Symbols that bespeak this similarity of status include using similar chairs for therapist and clients. They share their knowledge as team members and in the spirit of collaboration. When clients have not scheduled appointments for a while, therapists respect and support their autonomy. If feeling very concerned about a client's well-being, therapists call to schedule a session, expressing their own needs. Therapists' ultimate goal is to help clients thrive.

Primary Goal: Education

Therapists' main goal is to teach clients a way of thinking and a set of skills to improve mental health, cope with their challenges more effectively, and reach their goals. They do this through psychoeducation. When clients talk about a problem, they endeavor to address it and help solve it effectively. The session becomes problem focused. When clients depart from their stated goals, therapists redirect them.

THIS GOES ON THE RIGHT COLUMN

When clients talk about a problem, they endeavor to listen empathically to the clients experience before exploring solutions. Sessions are primarily experience focused.

Moralistic Assessments

Therapists think in terms of what is right and wrong regarding their clients' actions. If a client is having an extra-marital affair, they regard this as wrong.

Pathology Lens

Therapists view the people they serve as patients. Their symptoms are evidence of illness to be treated, and their unskilled behavior shows psychological or characterological deficiency.

Primary Goal: Therapeutic Environment

Therapists' main goal is to embody the therapeutic attitudes and create an environment conducive to self-connection, experiential process, healing, and improved mental health. Education is contextual and not assumed a priori. When clients talk about a problem, they endeavor to listen empathically before exploring solutions. When clients depart from their stated goals, therapists follow their lead.

Needs-Based Assessments

Therapists think in terms of values, another word for needs. If a client is having an extra-marital affair, they view this as lacking honesty and respect for their partner.

Unmet Needs Lens

Therapists view the people they serve as clients. Their symptoms are indicators of unmet needs, and their unskilled behavior shows lack of awareness.

Self-Judgment

When they make mistakes in therapy, therapists judge themselves harshly. They focus on the mistake and experience guilt and shame. They think of ways to restore wholeness to the situation and avoid making similar mistakes in the future.

Self-Empathy

When they make mistakes in therapy, therapists explore the feelings and needs underlying their actions, mourn the error, and think of ways to restore wholeness to the situation and avoid making similar mistakes in the future.

Expecting Compliance

When therapists make recommendations, they expect clients to cede to their authority and adhere to their recommendations. They might become irritated if they don't.

Respecting Autonomy

When therapists make recommendations, they assume that clients will make the final decision and support their choosing.

Making Demands

When therapists want clients to do something, they may make a demand, kindly. "Please call me 24 hours ahead when you're not going to come to the session. This would help me plan my day."

Making Requests

When therapists want clients to do something, they make requests. "Would you be willing to call me 24 hours ahead when you're not going to come to the session? This would help me plan my day."

Consequences

Therapists have a protocol of consequences if clients act in ways that impact them negatively. For example, clients who miss a session without cancelling ahead will be charged x amount. When clients do not adhere to the protocol, they discuss the issue with them; should the issue persist, they discuss termination as a consequence of the clients' actions.

Self-Care

When clients act in ways that impact therapists negatively, e.g., missing sessions without cancelling ahead, they discuss the issue with them in terms of therapists' unmet needs. Should the issue persist, they discuss termination based on therapists' need for consideration, self-care, and sustainability. Therapists have protocols for measures to be taken with regard to safety.

Being Right

When expressing their perspective regarding the clients' problems, therapists view their perception or interpretation as accurate, right, and true, and their suggestions as probably the optimal solution for clients.

If the clients' views differ, therapists refute the clients' perspectives without acknowledging that what they are expressing is their opinion.

Being Fallible

When expressing their perspective regarding the clients' problems, therapists are aware that their view may or may not be accurate, right, or true, and their suggestions may or may not be the optimal solution for clients.

If the clients' views differ, therapists acknowledge that what they are expressing is their opinion.

To be sure, there are times when we act as the authority in the relationship. For instance, when a client of mine was violent toward his family members, I hospitalized him against his wishes more than once. In a situation like this, we operate within the authority paradigm unequivocally, in the service of safety. Yet we want to do so judiciously, compassionately, and hopefully rarely.

SUMMARY

A number of attitudes govern the way ET practitioners engage with clients. Unconditional acceptance calls for accepting clients for who they are without conditions of “worthiness” and addressing situations that concern us without judgments. Empathy means understanding clients from their own frame of reference including their feelings, needs, meanings and wishes. Roadblocks to empathy include thinking about assessment, diagnosis, or intervention when listening. Witnessing is the act of being a caring observer of and acknowledging unfair situations experienced by clients. Genuineness is speaking and acting in ways that are in harmony with our thoughts and emotions. Warmth is our unspoken affection for clients conveyed through body language. Care refers to our interest in, and genuine concern for clients. Respect means treating clients with courtesy, consideration, and thoughtfulness, from being on time for sessions to protecting their private information, from wearing work-appropriate clothes to treating low-income and mandated clients with the same consideration we would their affluent and voluntary counterparts. Vulnerable honesty entails addressing issues with clients by offering a noninterpretive observation, revealing our feelings and needs, and making a concrete request. Unlike the authority paradigm, the partnership paradigm invites us to engage with clients collaboratively; this includes respecting clients’ meaning, asking authorization before offering our own perspective, answering questions directly, and supporting clients’ freedom of choice.